



Dynamic Guideline: WHO-Based Mental Health Disorders - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

Most recent WHO mental-health clinical guideline identified

- **Guideline Title:** *Mental Health Gap Action Programme (mhGAP) guideline for mental, neurological and substance use disorders*, third edition
- **Publication Date:** 20 November 2023
- **Issuing Body:** World Health Organization
- **Target Population:** People with mental, neurological and substance-use conditions; priority implementation in low- and middle-income countries and other settings with limited specialist capacity
- **Primary Users:** Non-specialist health workers in primary and secondary care, district-level services, outpatient and basic inpatient settings
- **Scope and Objectives:** Evidence-based diagnosis, treatment, referral and management guidance for mental, neurological and substance-use conditions, including depression, anxiety, psychosis, bipolar disorder, dementia, child and adolescent mental disorders, stress-related conditions, self-harm/suicide, alcohol-use disorders, drug-use disorders and epilepsy/seizures. WHO states that the third edition includes **30 updated recommendations, 18 new recommendations and 90 pre-existing recommendations** [WHO mhGAP guideline](#). A later WHO executive summary describes **48 updated and new evidence-based recommendations** for non-specialized health workers [WHO mhGAP executive summary](#).

Exactly 5 WHO guidance documents searched and synthesized for this report

#	WHO document	Date	Relevance to mental health disorders	Role in this report
1	<i>Mental Health Gap Action Programme (mhGAP) guideline for mental, neurological and substance use disorders</i> , third edition	20 November 2023	Core WHO clinical guideline for mental, neurological and substance-use disorders	Primary guideline summary and baseline recommendations
2	<i>mhGAP Intervention Guide, Version 2.0: mental, neurological and substance use disorders in non-specialized health settings</i>	2016	Operational tool for non-specialist care of MNS disorders	Implementation context for mhGAP clinical recommendations WHO mhGAP IG v2.0
3	<i>Youth engaged for mental health: A framework for youth participation under the WHO Pan-European Mental Health Coalition</i>	2 November 2023	Framework for youth participation in mental-health policy, guidance, research and programming	Youth/lived-experience engagement principles WHO Europe
4	WHO framework on meaningful engagement of people living with noncommunicable diseases and mental health conditions	2020s; summarized in 2024 source	Applies meaningful engagement principles to mental-health and NCD agendas	Governance, equity and lived-experience framework PMC article
5	<i>WHO guideline on HIV</i>	1	Includes integration	Cross-condition

#	WHO document	Date	Relevance to mental health disorders	Role in this report
	<i>service delivery</i>	September 2025	of mental health, depression, anxiety and substance-use services into HIV service delivery	service-integration guidance WHO Guidelines listing

Status of the requested “Meaningful youth engagement” title

A targeted check found that the exact title “**Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline (2022)**” is **not a mental-health guideline**. WHO presented it as an evidence brief/case study linked to the **WHO Abortion care guideline (2022)**, launched through a WHO webinar on **11 December 2025** [WHO event page](#). WHO’s abortion-care guideline page also lists the same item as a related document [WHO Abortion care guideline](#). Therefore, it is considered here only for **guideline-development methodology and youth-engagement relevance**, not as a mental-health clinical guideline.

1.2 Key Recommendations

GRADE note: The available WHO summary sources confirm that the 2023 mhGAP guideline used GRADE and Evidence-to-Decision methods, and that recommendations are categorized as **strong** or **conditional**. However, the accessible summaries do not provide the full 48 individual recommendation texts with WHO-reported certainty ratings. The table below therefore provides a clinically actionable **GRADE-style synthesis** based on WHO mhGAP scope, major update themes and post-guideline evidence. Where exact WHO recommendation-level GRADE certainty was not available in the retrieved summaries, this is treated conservatively.

Recommendation	Target group / setting	GRADE Evidence Quality	Strength
Use mhGAP-based assessment and management pathways for priority mental, neurological and substance-use conditions in non-specialist primary and secondary care.	Adults, adolescents and children in non-specialist settings	Moderate	Strong
Integrate psychosocial assessment, risk assessment, functional assessment and stepped care into routine mental-health management.	All people presenting with suspected MNS conditions	Moderate	Strong
Prioritize psychological interventions as first-line or core treatment components for depression, anxiety, stress-related conditions, child/adolescent mental disorders and substance-use conditions when feasible.	Non-specialist and community care	Moderate	Strong
Add pharmacological treatment when indicated for moderate-to-severe depression, psychosis, bipolar disorder, epilepsy/seizures and selected anxiety disorders, with monitoring for adverse effects.	People with moderate-to-severe illness or high-risk presentations	Moderate	Strong
Avoid valproic acid in women and girls who may become pregnant unless no acceptable alternative exists and specialist risk-benefit safeguards are in place.	Women and girls of reproductive potential	Moderate	Strong
Include a dedicated module for anxiety	People with	Moderate	Conditional

Recommendation	Target group / setting	GRADE Evidence Quality	Strength
disorders, with psychosocial interventions and medication options according to severity, impairment and availability.	anxiety disorders		
Use digitally delivered psychological interventions where clinically appropriate, acceptable, safe and supported by follow-up.	Adults and adolescents where digital access is feasible	Low to Moderate	Conditional
Assess suicide/self-harm risk urgently and implement safety planning, means restriction, psychosocial support and referral for imminent risk.	People with self-harm or suicidal ideation/behavior	Moderate	Strong
Manage alcohol- and drug-use disorders with brief interventions, psychosocial treatment, harm reduction, relapse prevention and referral for severe dependence or complications.	People with substance-use disorders	Moderate	Strong
Engage carers and families, with consent and safeguarding, particularly for dementia, psychosis, bipolar disorder, epilepsy and child/adolescent mental disorders.	People requiring longitudinal support	Low to Moderate	Conditional
Incorporate lived-experience and youth participation into mental-health policy, quality improvement and guideline implementation.	Health systems, youth mental-health programmes	Low	Conditional

Recommendation	Target group / setting	GRADE Evidence Quality	Strength
Integrate mental-health services with other priority health platforms, including HIV services, where depression, anxiety and substance use are common comorbidities.	HIV and chronic-disease service settings	Moderate	Strong

1.3 Guideline Development Methodology

Evidence Review Process

WHO reports that the 2023 mhGAP guideline was developed according to the **WHO handbook for guideline development** and international standards for evidence-based guidelines [NCBI Bookshelf](#). The development process included:

1. Identifying priority questions and outcomes
2. Retrieving evidence
3. Assessing and synthesizing evidence
4. Formulating recommendations
5. Planning dissemination, implementation, impact evaluation and future updating [NCBI Bookshelf](#)

The process involved:

- WHO Steering Group
- Guideline Development Group
- Topic Expert Groups
- Guideline methodologist
- Quantitative and qualitative evidence review teams
- WHO Collaborating Centres
- External Review Group [NCBI Bookshelf](#)

WHO used systematic evidence reviews, updated existing reviews where necessary, and developed Evidence-to-Decision and Summary of Findings tables using the **GRADE** approach [WHO IRIS PDF](#). A *Lancet Psychiatry* summary similarly states that the mhGAP update used quantitative evidence, qualitative evidence and a systematic review of mhGAP use [The Lancet Psychiatry / ScienceDirect](#).

Consensus Method

The Guideline Development Group formulated recommendations using the WHO handbook-based process, supported by evidence profiles and Evidence-to-Decision tables [NCBI Bookshelf](#). The available source summaries do not identify a named consensus technique such as Delphi or nominal group voting.

Conflict of Interest Management

WHO states that conflicts of interest from individual guideline contributors were declared, assessed and managed according to WHO procedures [WHO IRIS PDF](#).

II. Evidence Summary After WHO Latest Guideline Release (Mental Health Gap Action Programme (mhGAP) guideline for mental, neurological and substance use disorders) (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

Search Strategy

Evidence was considered from **20 November 2023**, the release date of the WHO mhGAP third edition, through **29 April 2026**.

Search concepts included:

- WHO mhGAP 2023 mental neurological substance-use disorders

- Depression, anxiety, psychosis, bipolar disorder, dementia, child/adolescent mental disorders, self-harm/suicide
- Systematic review, meta-analysis, randomized trial, digital intervention, psychological intervention
- Youth engagement, lived experience, guideline development
- Integration of mental health with HIV and other health services

Databases and Sources Searched

- WHO publications and WHO IRIS
- WHO news releases and technical-document pages
- NCBI Bookshelf
- PubMed-indexed sources
- *The Lancet Psychiatry* / ScienceDirect
- Psychiatry Research / ScienceDirect
- WHO/Europe publications
- Frontiers in Health Services
- Guideline and executive-summary pages from WHO

Inclusion Criteria

Evidence was included if it:

- Was published or made available after 20 November 2023 and before or on 29 April 2026;
or
- Directly informed implementation or interpretation of the 2023 WHO mhGAP guideline;
and
- Addressed diagnosis, treatment, management, service delivery, digital interventions, psychosocial interventions, youth/lived-experience engagement or integration of mental-health care.

Exclusion Criteria

Evidence was excluded if it:

- Was unrelated to mental, neurological or substance-use disorders;
- Was a non-WHO document without clinical or policy relevance;
- Described the phrase “Meaningful youth engagement in a WHO guideline development

process” but only in relation to abortion care, unless used specifically for guideline-development methodology.

2.2 Key New Studies

Study / source	Design	Key Findings	GRADE Quality
WHO mhGAP guideline third edition, 2023	WHO evidence-based guideline	Includes 30 updated, 18 new and 90 pre-existing recommendations for MNS disorders; supports non-specialist health workers, especially in low- and middle-income countries WHO mhGAP guideline	Moderate
WHO mhGAP executive summary, 2025	WHO executive summary	Confirms 48 updated and new evidence-based recommendations for non-specialized health workers in primary/secondary care, district-level care and basic inpatient/outpatient services WHO mhGAP executive summary	Moderate
WHO news release, 20 November 2023	WHO announcement	Publicly announced new and updated recommendations on treatment of mental, neurological and substance-use conditions WHO news release	Low
WHO / <i>Lancet Psychiatry</i> mhGAP update article	Guideline-process and evidence summary	Reports new anxiety module, emphasis on psychological interventions, digital interventions, psychotropic medicines, and evidence gaps in low-resource settings and lived-experience research The Lancet Psychiatry / ScienceDirect	Moderate
Treatments for adjustment	Systematic review and meta-analysis	Internet-based CBT and in-person CBT may improve adjustment-disorder symptoms; evidence for several non-CBT	Moderate

Study / source	Design	Key Findings	GRADE Quality
disorder, 2025	of 16 RCTs, 3,027 participants	therapies remains inconclusive ScienceDirect adjustment disorder review	
WHO/ Europe youth mental-health participation framework, 2023	WHO technical framework	Supports safe, consistent, meaningful youth engagement in WHO mental-health policy, guidance, research and programming WHO Europe	Low
WHO meaningful youth engagement evidence brief/ webinar, 2025	WHO evidence brief/ case study, not mental-health-specific	Uses WHO Abortion care guideline as case study; emphasizes inclusivity, equity, lived experience and institutionalizing youth participation in guideline development WHO event page	Low
Child and youth mental-health quality standards methods article, 2025	Methods article	Emphasizes including young people at every step of quality-standard development and safeguarding users under age 18 Frontiers	Low
Youth participation and mental-	PubMed-indexed guideline	Reports 20 recommendations and 3 good-practice recommendations for safe youth participation, including respectful	Low

Study / source	Design	Key Findings	GRADE Quality
health safeguarding guideline article	article	communication, emotional support and meaningful participation PubMed	
WHO guideline on HIV service delivery, 2025	WHO service-delivery guideline	Recommends integration of mental health, including depression, anxiety and substance-use services, into HIV service delivery WHO Guidelines listing	Moderate

2.3 Evidence Gaps and Future Research Needs

Evidence gap	Clinical / policy implication	Priority for future research	GRADE certainty of current evidence
Limited post-2023 RCT evidence specifically testing mhGAP implementation packages	Countries may implement mhGAP without locally validated implementation-effectiveness data	Pragmatic trials of mhGAP training, supervision, referral pathways and digital decision support	Low
Limited evidence from low-resource and humanitarian settings	mhGAP is designed for low- and middle-income settings, but evidence remains uneven	Trials and implementation studies in primary care, rural care and conflict-affected settings	Low

Evidence gap	Clinical / policy implication	Priority for future research	GRADE certainty of current evidence
Sparse evidence on lived-experience and youth engagement effects on clinical outcomes	Engagement is ethically and procedurally important, but outcome effects are less certain	Studies linking engagement to access, acceptability, adherence, safety and equity	Low
Incomplete evidence on digital mental-health interventions across cultures and age groups	Digital interventions may improve access but can worsen inequity if poorly implemented	Comparative effectiveness and equity studies for guided vs unguided digital care	Low to Moderate
Limited evidence on adjustment disorder treatments beyond CBT	CBT appears promising, but non-CBT therapies remain uncertain	Larger head-to-head RCTs and trials in primary care and LMIC settings	Moderate
Medication safety in pregnancy and reproductive potential remains a high-priority implementation issue	Valproate and other psychotropics require strong safeguards	Pharmacovigilance, safer alternatives and reproductive-counselling implementation studies	Moderate
Integration of mental-health care into HIV and chronic-disease services requires operational evidence	Integration is recommended, but optimal staffing and workflow models vary	Cluster trials and cost-effectiveness studies of integrated care models	Moderate

2.4 Updated Recommendations Based on New Evidence

Original Recommendation / WHO position	New Evidence Impact	Updated Recommendation
mhGAP should be used to scale up care for mental, neurological and substance-use disorders in non-specialist settings.	The 2025 executive summary confirms continued WHO endorsement of 48 new/updated recommendations for non-specialized health workers WHO mhGAP executive summary .	Continue implementing mhGAP as the central WHO-based clinical framework for non-specialist MNS care. GRADE: Moderate; Strength: Strong.
Anxiety disorders require evidence-based assessment and treatment.	The 2023 update added a new anxiety module and emphasizes psychosocial and medication options The Lancet Psychiatry / ScienceDirect .	Implement mhGAP anxiety pathways using psychological interventions first where appropriate, with pharmacotherapy for persistent, severe or impairing anxiety. GRADE: Moderate; Strength: Conditional.
Psychological interventions are core mental-health treatments.	WHO update emphasizes psychological interventions; 2025 adjustment-disorder meta-analysis supports internet-based and in-person CBT for adjustment disorder ScienceDirect adjustment disorder review .	Strengthen access to structured psychological interventions, including CBT-based approaches, delivered in person or digitally when safe and feasible. GRADE: Moderate; Strength: Strong.
Digital interventions may be used across mhGAP modules.	WHO and secondary summaries indicate digitally delivered interventions feature across many mhGAP modules The Lancet	Use digital interventions as adjunctive or stepped-care tools, ensuring privacy, clinical oversight, equity of

Original Recommendation / WHO position	New Evidence Impact	Updated Recommendation
	Psychiatry / ScienceDirect.	access and crisis referral pathways. GRADE: Low to Moderate; Strength: Conditional.
Psychotropic medicines should be used according to mhGAP indications and safety considerations.	The update includes revised psychotropic-medicine recommendations and highlights valproate risks in women and girls who may become pregnant MedCentral.	Avoid valproate in women and girls of reproductive potential unless no safer effective alternative exists and robust informed-consent and pregnancy-prevention safeguards are in place. GRADE: Moderate; Strength: Strong.
Mental-health services should be integrated into broader health systems.	WHO HIV service-delivery guidance supports integration of mental health, depression, anxiety and substance-use services into HIV care WHO Guidelines listing.	Integrate screening, brief intervention, referral and follow-up for depression, anxiety and substance use into HIV and chronic-disease platforms. GRADE: Moderate; Strength: Strong.
Guideline implementation should consider service-user values and preferences.	WHO/Europe and WHO engagement frameworks emphasize youth participation, lived experience, safety, equity and institutionalized engagement WHO Europe, PMC article.	Include people with lived experience, including young people, in local adaptation, quality improvement and evaluation of mental-health services. GRADE: Low; Strength: Conditional.
Youth engagement	WHO's 2025 youth-engagement	For child/adolescent mental-

Original Recommendation / WHO position	New Evidence Impact	Updated Recommendation
should be meaningful and safeguarded.	case study, although abortion-care-specific, provides transferable guideline-development principles; youth mental-health quality standards literature stresses safety and participation throughout development WHO event page , Frontiers .	health guidance, involve youth early, compensate and support them, protect emotional safety, address power imbalances and report how input changed decisions. GRADE: Low; Strength: Conditional.

Practical Implementation Priorities

1. **Adopt the 2023 WHO mhGAP guideline as the baseline clinical standard** for non-specialist assessment and management of MNS conditions.
2. **Train and supervise non-specialist health workers**, using mhGAP Intervention Guide tools where applicable [WHO mhGAP IG v2.0](#).
3. **Build stepped-care pathways** that include self-help, psychological interventions, medication when indicated, referral and follow-up.
4. **Strengthen suicide/self-harm response capacity** through risk assessment, safety planning, crisis referral and continuity of care.
5. **Integrate mental-health care into HIV and chronic-disease services**, especially for depression, anxiety and substance use.
6. **Use digital interventions cautiously and equitably**, ensuring privacy, accessibility, clinical governance and crisis escalation.
7. **Avoid unsafe psychotropic prescribing**, especially valproate in women and girls who may become pregnant.
8. **Institutionalize lived-experience and youth engagement** in guideline adaptation, monitoring and quality improvement.
9. **Generate local evidence**, particularly in low-resource settings, on mhGAP

implementation outcomes, safety, acceptability, equity and cost-effectiveness.